

Orthotopic Neobladder

WARWIKI

Patient Instructions · A new bladder, built from bowel, that you urinate through

An orthotopic neobladder is a new bladder built from a piece of your own bowel and connected to your urethra, so you can urinate the natural way — with no bag — after the bladder is removed or no longer works. It takes time and practice to learn to empty it well and to regain control.

About This Procedure

About **16–24 inches of small intestine** is opened and refolded into a low-pressure pouch, then connected to your **urethra**; the ureters (tubes from the kidneys) drain into it. There is **no stoma and no bag**.

Because the new bladder is made of bowel, it does **not** give the same “got-to-go” signal, so you learn to:

- Empty on a **schedule** — by relaxing your pelvic floor and gently bearing down — including **waking at night**.
- Use a catheter to drain it at times if it does not empty fully (more common in women).
- Do **pelvic-floor exercises** to build control.

Control improves over **months**; some leakage, especially at night, is common early on. Not everyone is a candidate — it depends on your urethra, kidney function, and ability to manage the routine.

Is It Safe?

It is a well-established option that gives the most “natural” result. It is a larger, longer operation than a conduit and needs more learning and aftercare. Risks include surgery risks (bleeding, infection, a leak), daytime or nighttime leakage, incomplete emptying needing a catheter, mucus, urine infections, and long-term salt/vitamin/kidney changes watched at follow-up.

LEARN THE TERMS

Neobladder

A new bladder made from a piece of bowel.

Orthotopic

Placed in the normal spot and joined to the urethra, so you urinate the usual way.

Urethra

The tube urine passes through to leave the body.

Ureters

The tubes that carry urine from the kidneys; they drain into the neobladder.

Timed voiding

Urinating on a set schedule (day and night) rather than waiting for an urge.

Self-catheterization

Passing a thin tube yourself to fully empty the neobladder when needed.

Pelvic floor

The muscles you train to help hold and release urine.

Mucus

Slippery fluid the bowel makes; strands in the urine are normal.

WILL IT HURT? The surgery is done under general anesthesia, so you feel nothing during it. Afterward, expect belly soreness, and the bowel takes a few days to “wake up.” You wake up with a catheter in the new bladder (it stays about 2–3 weeks) plus soft stents. Most people stay in the hospital about 5–8 days; the surgery itself is long.

How to Get Ready (Before Surgery)

- Done under **general anesthesia** — follow all surgery instructions (fasting, hold blood thinners as told). You may have a **bowel prep**.
- Understand the commitment: **timed voiding day and night**, possible **self-catheterization**, and pelvic-floor exercises.
- **Do not smoke**, and arrange help at home afterward.

Tell your team ahead of time if you:

- Take a **blood thinner**, or have a current infection
- Have **bowel disease** or kidney problems, or prior abdominal surgery or radiation

What Happens During Surgery

- 1 You are asleep under anesthesia and antibiotics are given.
- 2 A piece of bowel is set aside and the rest of the bowel is reconnected.
- 3 The bowel is opened and folded into a pouch, then sewn to the urethra.
- 4 The ureters are connected to the pouch; a catheter and soft stents are placed while it heals.

After Surgery

- You go home in about **5–8 days** with a **catheter for ~2–3 weeks**; you may flush it to clear mucus.
- A **healing X-ray** of the neobladder is done before the catheter is removed.
- Then you learn **timed voiding** (set an alarm at night), pelvic-floor exercises, and **self-catheterizing** if it does not empty fully.
- Expect **leakage early** (especially at night) that improves over months. Mucus is normal; drink fluids; lifelong follow-up (incl. vitamin B12).

Call your care team or seek care if you have:

- A fever or chills
- You **cannot urinate** or the catheter **stops draining**
- Severe belly pain, vomiting, or **no gas or stool**
- Heavy bleeding, wound redness or drainage, or flank pain

THREE THINGS TO REMEMBER

1. A neobladder is a new bladder made from bowel and joined to your urethra, so you urinate the natural way with no bag.
2. It needs practice: timed voiding day and night, pelvic-floor exercises, and sometimes self-catheterizing. Control improves over months and early leakage is normal.
3. Prepare by following fasting and bowel-prep instructions and not smoking. Keep the catheter until your team removes it after a healing X-ray, and call right away if you cannot urinate or the catheter stops draining.